

NORTHGATE MEMORIAL HOSPITAL

INPATIENT DISCHARGE SUMMARY

Department of Internal Medicine | Clinical Research Unit — Ward 7B

PATIENT DEMOGRAPHICS

Patient Full Name: Robert James Whitmore

Date of Birth: 07 September 1951 | Age: 73 years | Sex: Male

Address: Flat 4B, 32 Cavendish Road, Manchester, M20 4NL, United Kingdom

NHS Number: 943 867 2210 | **Hospital MRN:** NMH-8834521

Phone: 0161-555-0147 | Next of Kin: Patricia Whitmore (wife), 0161-555-0288

Private Insurance Policy: BUPA UK — Policy Number BCE-9977-2031-X | Expiry: 31/12/2025

GP: Dr. Amara Singh, Didsbury Medical Centre, 88 Palatine Road, Manchester M20 3JA

Study Subject ID: SUB-031 | Study: ZENITH-001 | Site: Northgate Memorial Hospital — Site 009

ADMISSION DETAILS

Date of Admission: 14 September 2024 | Date of Discharge: 19 September 2024 | Length of Stay: 5 days

Admitting Diagnosis: Acute hepatic decompensation, nausea and vomiting, dehydration

Admitting Physician: Dr. Nadia Obukhova, Consultant Hepatologist

Study-Related Admission: YES — Suspected treatment-related hepatotoxicity in context of ZENITH-001 participation

REASON FOR ADMISSION AND PRESENTING COMPLAINT

Mr. Whitmore, a 73-year-old male participant in the ZENITH-001 Phase II clinical trial, presented to the Emergency Department on 14 September 2024 with a 5-day history of worsening nausea, vomiting (6–8 episodes per day), profound fatigue, anorexia, and dark urine. His wife reported he appeared jaundiced on the morning of admission. He had been unable to eat solid food for 4 days prior to presentation.

It is noted that Mr. Whitmore missed his scheduled protocol study visit on 15 September 2024 (Cycle 4 Day 15 safety assessment) due to this hospitalisation. The missed visit was not prospectively communicated to the investigative site prior to the admission date.

MEDICAL HISTORY AND BACKGROUND

Oncological: KRAS G12C-mutant non-small cell lung carcinoma (NSCLC), stage IIIB, diagnosed February 2022. Prior systemic treatment includes carboplatin-paclitaxel chemotherapy (4 cycles, 2022) and pembrolizumab immunotherapy (18 cycles, 2022–2024). Currently enrolled in ZENITH-001 since June 2024 (Cycle 1 Day 1: 03 June 2024). Best radiological response to date: partial response (PR) assessed at Cycle 2 evaluation.

Comorbidities: Type 2 diabetes mellitus (diet-controlled), benign prostatic hyperplasia, hypertension, chronic kidney disease Stage 2 (baseline creatinine 1.3 mg/dL).

MEDICATION RECONCILIATION ON ADMISSION

Per patient and wife's account and pharmacy records, Mr. Whitmore has been fully compliant with all prescribed study and non-study medications throughout his participation in ZENITH-001. He takes his study drug every morning without fail and has not missed a dose.

However, during the ward admission on Day 2, Mr. Whitmore disclosed to the ward nurse that he stopped taking the study medication (Veridaxinib) approximately 3 weeks prior to admission, on approximately 24 August 2024, because he 'felt something was wrong with his liver.' He did not inform the study team of this self-discontinuation and did not contact the site between visits.

Non-study medications on admission: Tamsulosin 0.4 mg daily, Amlodipine 5 mg daily, Aspirin 75 mg daily, Paracetamol 500 mg as needed. No herbal supplements or over-the-counter medications reported initially; wife later noted he had been taking 'liver detox' herbal supplement capsules purchased online for 6 weeks.

ADMISSION LABORATORY FINDINGS

Test	Day 1 (Admission)	Day 3	Day 5 (Discharge)	Ref Range
ALT	1,241 U/L	876 U/L	312 U/L	10–40 U/L
AST	987 U/L	654 U/L	287 U/L	10–40 U/L
Total Bilirubin	4.8 mg/dL	3.2 mg/dL	2.1 mg/dL	0.2–1.2 mg/dL
Direct Bilirubin	3.1 mg/dL	2.0 mg/dL	1.4 mg/dL	0.0–0.3 mg/dL
Alkaline Phosphatase	312 U/L	287 U/L	241 U/L	40–150 U/L
INR	1.7	1.4	1.2	0.9–1.1
Creatinine	1.9 mg/dL	1.6 mg/dL	1.4 mg/dL	0.7–1.3 mg/dL
Haemoglobin	10.2 g/dL	10.5 g/dL	10.6 g/dL	13.5–17.5 g/dL

CLINICAL COURSE AND MANAGEMENT

Mr. Whitmore was admitted to Ward 7B and commenced on IV fluid resuscitation (0.9% NaCl 125 mL/hr), IV N-acetylcysteine infusion per hepatology protocol, and antiemetic therapy with ondansetron IV and metoclopramide. Hepatology and Gastroenterology were consulted. Liver ultrasound showed no biliary obstruction, no focal lesions. Autoimmune hepatitis serology was negative. Hepatitis A, B, C, and E serology negative. CMV and EBV PCR negative. Abdominal CT showed no evidence of disease progression.

Over the 5-day admission, liver function tests showed a gradual improving trajectory consistent with drug-induced liver injury (DILI) in the context of Veridaxinib exposure and possible herbal supplement contribution. No serious adverse events occurred during this hospital admission. INR normalised without vitamin K supplementation.

DISCHARGE DIAGNOSIS

- 1. Drug-induced liver injury (DILI), Grade 4 by CTCAE v5.0, likely related to Veridaxinib (VDX-114) — study drug permanently discontinued.
- 2. Acute kidney injury, Grade 2, superimposed on CKD Stage 2 — resolving at discharge.
- 3. Missed study protocol visit (15 September 2024) — Protocol Deviation Report to be filed by investigator.

DISCHARGE MEDICATIONS

Veridaxinib PERMANENTLY DISCONTINUED. Ursodeoxycholic acid 500 mg twice daily for 8 weeks. All other prior medications resumed per pre-admission doses.

Discharged to home care with follow-up arranged at oncology clinic in 7 days.

Discharge Summary completed by: Dr. Nadia Obukhova, Consultant Hepatologist
Electronically countersigned: Dr. Marcus Webb, Principal Investigator, Site 009
Date: 19 September 2024 | Time: 14:22 BST